

TURP (Transurethral Resection of the Prostate)

Patient Information · The long-standing standard surgery for an enlarged prostate

WARWIKI

Transurethral resection of the prostate (TURP) is a common, well-tested surgery for an enlarged prostate (BPH) that blocks the flow of urine. The surgeon works through a scope passed up the urethra, so there is **no cut on the skin**. The inner prostate tissue that squeezes the urine channel is trimmed away. Most men get a stronger stream that lasts for years.

About This Procedure

A thin scope (a **resectoscope**) is passed through the urethra. A small electric wire loop removes the blocking tissue piece by piece. This widens the channel. The outer part of the prostate stays in place. A catheter drains the bladder for a few days while it heals.

Benefits and Risks in Numbers

9 in 10

men do not need another prostate operation within 5 years

6–7 in 10

have little or no semen afterward (harmless)

2–10 in 100

later get scar tissue that narrows the urine channel

Numbers are averages from published studies. Your own risk may differ; ask your team.

Is It Right for Me?

TURP works best for small to medium prostates. Very large prostates may be better treated with HoLEP or simple prostatectomy. Aquablation has also been used, with less evidence for very large glands. Like any surgery, it has risks: bleeding, infection, and trouble urinating at first.

Learn the Terms

TURP

Transurethral resection of the prostate. Scope surgery for BPH.

BPH

An enlarged prostate that is not cancer.

Resectoscope

The scope with a wire loop used to trim the prostate.

Retrograde ejaculation

Semen goes back into the bladder instead of out. It is common and harmless.

Catheter

A soft tube that drains the bladder for a few days after surgery.

Bladder irrigation

Gentle rinsing through the catheter to clear blood early on.

What changes afterward? Most men get a much stronger stream. The most common lasting change is **retrograde ejaculation**: little or no fluid comes out. You usually still have an orgasm, though it may feel a little different. It is harmless, but it can make it harder to father a child. Most men notice no change in erections, and lasting leakage is rare. New leakage is more likely after past radiation to the prostate.

First 1–3 days Catheter, sometimes with rinsing. You may stay one or more nights.	First few weeks Pink urine, burning and urgency. Drink fluids. No straining or heavy lifting.	Weeks 1–2 A small bleed is common as it heals. Rest and drink more.	Follow-up visit Your team checks your flow and how well you empty.
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How to Get Ready

- Follow your instructions about eating, drinking and which medicines to stop, including blood thinners.
- A **urine test** checks for infection first.
- Arrange a ride home.

What Happens

- 1 You get **spinal or general anesthesia**, so you feel nothing, and antibiotics.
- 2 The blocking tissue is trimmed through the scope.
- 3 A catheter is placed. The bladder may be rinsed for the first day.
- 4 The catheter usually comes out in **1–3 days**.

If your stream gets weak again months or years later, tell your team. Scar tissue can be treated.

Call the office that did your surgery if you have:

- A temperature of **100.4 °F (38 °C)** or higher, or chills
- Bleeding or burning that gets worse instead of better

Go to an emergency department if:

- You cannot urinate for several hours and your bladder feels full or painful
- Your urine looks like red wine or has clots, or the catheter stops draining
- You cannot reach the office and are worried

Three Things to Remember

1. TURP removes the blocking prostate tissue through a scope, with no cut on the skin.
2. Expect a stronger stream. Little or no semen afterward is common and harmless.
3. Pink urine and burning settle over a few weeks.